

DR. SHALINI BHARGAVA

M.B.B.S, D.G.O
Consultant Obstetrician
and gynaecologist
Senior Infertility Specialist

डॉ. शालिनी भार्गव

M.B.B.S, D.G.O
स्त्री रोग विशेषज्ञ
निःसंतानता विशेषज्ञ

Name Shanti s/o Rajenderji Kohari Age 19y Sex MF Date 22/10/2025

R

wt = 42 kg

ht = 5' 3"

AI
itfb

CBC

TSH, T₃, T₄

LH

FSH

s. Prolactin

usg for uterus and
adnexa to rule
out any pathology

Shanti

6 days x 2 yrs
42-45

3-4
2der

Menarche at 12y

LMP = 10/09/2025

AI

Tab Regesteron 2mg
1 — 1 x 3 days

9784400368

पता : 16, पंचवटी
आर.के. मॉल के पीछे, उदयपुर

मिलने का समय :
ग्रीष्म :- 7-8 AM / 7-8 PM
शीतकालीन :- 8-9 AM / 6-7 PM

Ph. No. : 9784400368
~~0294-2426484~~

Add. : 16, Panchvati, Behind, R.K. Mall, Udaipur (Raj.)



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9351910512



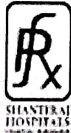
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REG NO.		NAME		BILL NO.		AGE		CONSULTANT		CATEGORY		ADDRESS		DATE		FATHER/HUSBAND		RECEIPT NO.		GENDER		DEPARTMENT		Mobile No	
UHID19633		MS. JANVI KOTHARI		OB047905		19 Y		Dr. SHALINI BHARGAV		GENERAL		JYOTI NAGARUDAIPUR		:27-10-2025/ 8:49AM		: RAJENDRA KOTHARI		: OR58740		: Female		: GYNAECOLOGIST		: 9414357197	
S.No.		Particulars		Performed By		Qty		Amount(Rs.)																	
1		VITAMIN B12				1.00		900.00																	
2		VITAMIN - D3				1.00		1600.00																	
								Total		2500.00															
								DISCOUNT		400.00															
								Net Amount		2100.00															
								Received		2100.00															
PAYMENT MODE : CASH																									
Two Thousand One Hundred Rupees only																									
Invoice Remarks :BY RASHMI MAAM																									
																						Authorised Signatory			
																						GOURAV SALVI			



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UHID19633		MS. JANVI KOTHARI		OB047675		19 Y		Dr. SHALINI BHARGAV		GENERAL		JYOTI NAGARUDAIPUR		:22-10-2025/ 1:30PM		: RAJENDRA KOTHARI		: OR58458		: Female		: GYNAECOLOGIST		: 9414357197	
S.No.		Particulars		Performed By		Qty		Amount(Rs.)																	
1		USG Pelvis				1.00		800.00																	
								Total		800.00															
								DISCOUNT		100.00															
								Net Amount		700.00															
								Received		700.00															
PAYMENT MODE : CASH																									
Seven Hundred Rupees only																									
Invoice Remarks :DISC. BY RASHMI MAM																									
																						Authorised Signatory			
																						JITENDRA MEENA			



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UHID19633		MS. JANVI KOTHARI		OB047904		19 Y		Dr. SHALINI BHARGAV		GENERAL		JYOTI NAGARUDAIPUR		:27-10-2025/ 8:21AM		: RAJENDRA KOTHARI		: OR58737		: Female		: GYNAECOLOGIST		: 9414357197	
S.No.		Particulars		Performed By		Qty		Amount(Rs.)																	
1		COMPLETE BLOOD COUNT (CBC)				1.00		350.00																	
2		TFT T3 T4 TSH				1.00		600.00																	
3		SERUM PROLACTIN				1.00		650.00																	
4		LH				1.00		650.00																	
5		FSH				1.00		600.00																	
								Total		2850.00															
								DISCOUNT		350.00															
								Net Amount		2500.00															
								Received		2500.00															
PAYMENT MODE : CASH																									
Two Thousand Five Hundred Rupees only																									
Invoice Remarks :BY RASHMI MAAM																									
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																						GOURAV SALVI			
																						Authorised Signatory			



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Name: Ms. Janvi Kothari	Age: 19 yr.	Sex: F	Date: 23/10/2025
Ref. By Dr. Shalini Bhargav	Patient No. 19633		

ULTRASOUND STUDY OF PELVIS

Uterus: appears normal in position. It appears normal in size 6.7 x 2.8 x 3.8 cm and normal echotexture.

Endometrial thickness appears normal, measures 10 mm.

Right ovary: appears normal in size measures: 25 x 10 x 25 mm, volume : 3.6 cc, shape and echotexture.

A hemorrhagic cyst of size 26 x 23 x 29 mm, volume: 9.6 cc is seen in left ovary

No free fluid is seen in POD.

IMPRESSION:

❖ **Left ovarian hemorrhagic cyst.**

Advise - Clinical correlation and relevant further workup is suggested.


Dr Kapil Aggarwal (DNB)
Consultant Radiologist
RMC 41837/17040



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Patient Name : MS JANVI KOTHARI



SHP25/6401

Age/Gender : 19 Yrs/ Female

Registration Date : 27/10/2025 08:40 AM

Ref. Dr. : Dr. SHALINI BHARGAV

Collection Date : 27/10/2025 08:42 AM

Center : SHANTIRAJ HOSPITAL PVT LTD

Report Date : 27/10/2025 03:35 PM



HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
COMPLETE BLOOD COUNT			
Haemoglobin	14.3	gm/dL	12.0 - 15.0
Total WBC Count	8890	cell/cu.mm	4000 - 10000
RBC Count	4.60	mil/cu.mm	4.5 - 5.5
RBC INDICES			
Hematocrit HCT	41.0	%	40 - 50
Mean Corp Volume MCV	89.0	fL	83 - 101
Mean Corp Hb MCH	31.1	pg	27 - 32
Mean Corp Hb Conc MCHC	35.0	gm/dL	31.5 - 34.5
RDW-CV	12.8	%	11.6 - 14.0
RDW-SD	43.1	fl	35.0 - 56.0
Platelet Count	2.01	10 ³ /uL	1.50 - 4.50
Platelet Indices			
MPV	11.2	fL	9.0 - 13.0
PDW-CV	16.1	%	14 - 17
DIFFERENTIAL LEUCOCYTE COUNT			
Neutrophils	74	%	40 - 80
Lymphocytes	18	%	20 - 40
Monocytes	07	%	02 - 10
Eosinophils	01	%	01 - 06
Basophils	0	%	01 - 02

NOTE:

- As per the recommendation of International council for Standardization in Hematology, the differential leukocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
- Test conducted on EDTA whole blood.

**** End of the report. ****

Checked By

Manish

Dr Manish Mewani
MBBS MD Pathology

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Name Mrs. JAWVI KOTHARI

Visit Date & Time 27/10/2023 10:07:52

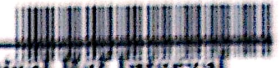
PATIENT ID 102519142

Age 39 Yrs Sex Female

Sample Accepted : 27/10/2023 10:12:08

Test Authenticated at : 27/10/2023 11:09:00

Ref By Dr. SHALINI BHARGAVA



Test Name	Value	Status	Unit	Biological Ref Interval
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THYROID PROFILE

T3	1.43		ng/ml	0.75 - 2.10
T4	8.37		ug/dl	5.00 - 13.00
TSH	3.090		micro U/ml	0.340 - 5.600

TSH levels may be affected by acute illness and drugs like dopamine and glucocorticoids.

Low or undetectable TSH is suggestive of Grave's disease

TSH between 5.5 to 15.0 with normal T3,T4 indicates impaired thyroid hormone or subclinical hypothyroidism or normal T3,T4 with slightly low TSH suggests subclinical hyperthyroidism

TSH suppression does not reflect severity of hyperthyroidism therefore, measurement of FT3,FT4 is important.

Free T3 is first hormone to increase in early hyperthyroidism.
Only TSH level can prove to be misleading in patients on treatment.

Therefore Free T3, Free T4 along with TSH should be checked.
During pregnancy clinically T3T4 can be high and TSH can be slightly low.



(Signature)

DR. NARENDRA MOGRA
RMC No. 5398

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Age 19 Yrs Sex Female

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Ref. By Dr. SHALINI BHARGAVA



IMMUNOLOGY

Test Name

Value

Status

Unit

Biological Ref Interval

LH

11.30

miU/ml

Follicular Phase: 1.6-12.18
miU/ml
Ovulatory Phase: 7.31-92.53
miU/ml
Luteal Phase : 0.5-15.1
miU/ml
Post Menopausal
: 5.04-63.11 miU/ml
Male -0.5-12.43



[Signature]

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Age 19 Yrs Sex Female	Sample Accepted : 27/10/2025 10:12:08	
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IMMUNOLOGY

Test Name	Value	Status	Unit	Biological Ref Interval
PROLACTIN	27.20		ng/ml	Men : 2.64-13.13 ng/ml Woman : (Premenopausal) 13.34-26.72 ng/ml (Postmenopausal) : 2.74-19.64 ng/ml



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RMC No. 5399

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Ref. By Dr. SHALINI BHARGAVA



IMMUNOLOGY

Test Name	Value	Status	Unit	Biological Ref Interval
VITAMIN B12	175.00	L	PG/ML	192.00 - 827.00

Nutritional and macrocytic anemia's can be caused by a deficiency of vitamin B12. This deficiency can result from diets devoid of meat and bacterial products, from alcoholism, or from structural/functional damage to digestive or absorptive processes (forms of pernicious anemia). Malabsorption is the major cause of this deficiency through pancreatic deficiency, gastric atrophy or gastrectomy, intestinal damage, loss of intestinal vitamin B12 binding protein (Intrinsic factor), production of auto antibodies directed against intrinsic factor, or related causes. This vitamin is necessary for normal metabolism, DNA synthesis and red blood cell regeneration. Untreated deficiencies will lead to megaloblastic anemia and vitamin B12 deficiency results in irreversible central nervous system degeneration. Vitamin B12 or folate are both of diagnostic importance for the recognition of vitamin B12 or folate deficiency, especially in the context of the differential diagnosis of megaloblastic anemia. Radio assays were first reported for vitamin B12 in 1961. All utilize co-Cyanocobalamin radiolabeled fractions and intrinsic factor for binding vitamin B12. The various commercial assays differ in their free versus bound separation techniques and choice of specimen pretreatment. The presence of endogenous serum binding protein for Cyanocobalamin (transcobalamins including R-protein) and of immunoglobulin directed against intrinsic factor require that specimens are either boiled or treated at an alkaline pH to release the vitamin B12 and destroy the binding protein. In the late 1970's, radioassays using serum binding protein or partially purified intrinsic factor measured levels of vitamin B12 which exceeded those determined by microbiological methods. This was caused by the presence of the serum binding protein or R-protein in the assay. R-protein specificity is poor compared to that of intrinsic factor and vitamin B12 analogs were being measured in addition to vitamin B12 itself. Since that time, recommendations have been established for the use of highly purified intrinsic factor throughout the industry. Roche Cobas vitamin B12 employs a competitive test principle using intrinsic factor specific for vitamin B12. Vitamin B12 in the sample competes with the added vitamin B12 labeled with biotin for the binding sites on the ruthenium-labeled intrinsic factor complex.**

METHOD : ELECTRO CHEMILUMINESCENCE ASSAY



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IMMUNOLOGY

Test Name	Value	Status	Unit	Biological Ref Interval
VITAMIN D3	8.52		NG/ML	Deficient- < 10.0 ng/ml Insufficient - 10-29 ng/ml Sufficient -30-100 ng/ml Upper safety limit ->100 ng/ml

Vitamin D is a fat-soluble steroid hormone precursor that is mainly produced in the skin by exposure to sunlight or it is supplied via dietary sources (mainly egg yolk, fish oil and plants). The two most important forms of vitamin D are vitamin D3 (Cholecalciferol) and vitamin D (ergocalciferol). In contrast to vitamin D3, vitamin D2 has to be taken up with food. 25 -OH vitamin D is the metabolite that should be measured in blood to determine the overall vitamin D status because it is the major storage form of vitamin D in the human body. More than 95 % of 25-OH vitamin D, measurable in serum, is 25 -OH vitamin D3. Vitamin D deficiency is a common cause of secondary hyperthyroidism, osteomalacia, increased bone turnover, reduced bone mass and risk of bone fracture. Low 25 -OH vitamin D concentrations are also associated with lower bone mineral density. In conjunction with other clinical data, the results may be used as an aid in the assessment of bone metabolism. The Access-2 vitamin D3 (25-OH) assay employs a polyclonal antibody directed against vitamin D3

METHOD : FULLY AUTOMATED ELECTRO CHEMILUMNESCEENCE ASSAY



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IMMUNOLOGY

Test Name	Value	Status	Unit	Biological Ref Interval
FSH	3.08		miU/ml	Female (Mid Follicular Phase) : 3.30-12.9 miU/ml Mid Cycle Paek : 4.54-22.51 miU/ml Mid Luteal Phase : 1.5 -7.8 miU/ml Post Menopausal : 25.4 -135.6 miU/ml Male -1.4-12.6

*** End of Report ***



[Signature]

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